

CLL Objection Handler

BRUKINSA (zanubrutinib) · Gulf field team. 37 physician objections, each with its approved response, the supporting data points, and the trial, source file and slide it was taken from.

Internal draft. Not MRC approved.

Do not use with HCPs until reviewed. For internal review only — not for distribution outside BeOne.

Generated 5 September 2026

37 objections across 7 categories

Companion document: GAP_REPORT.md

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- 15 I am worried about atrial fibrillation. All the BTK inhibitors cause it.
- 16 What about hypertension? That is the adverse event I actually see in clinic.
- 17 If the patient is on it for years, they will eventually come off it for toxicity anyway.
- 18 Infections are what actually kill my CLL patients. What does BRUKINSA do to infection risk?
- 19 BRUKINSA causes more neutropenia than the alternatives.
- 20 My patient had to stop ibrutinib because of toxicity. Why would BRUKINSA be any different?

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- 21 BRUKINSA is not giving me extra clinical benefit over acalabrutinib. They are basically the same drug.
- 22 There is no head-to-head trial against acalabrutinib, so an indirect comparison does not prove anything.
- 23 Is there any evidence directly comparing BRUKINSA with acalabrutinib plus venetoclax?
- 24 What about venetoclax plus obinutuzumab? That is the fixed-duration regimen I actually use.
- 25 Acalabrutinib was non-inferior to ibrutinib in ELEVATE-RR. That is good enough for me.

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- 26 Why should I add BRUKINSA to the formulary when we already have acalabrutinib? Every addition costs the institution.
- 27 In my country the tender price for BRUKINSA sits above acalabrutinib, so the pharmacy committee will not approve it.
- 28 Continuous therapy means an open-ended budget commitment. Fixed duration lets me forecast.

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- 29 AV is the first and only approved all-oral, chemo-free therapy in 1L CLL, and it is finished in just over a year.
- 30 Twice a day is more of a burden for my patient than acalabrutinib's schedule.
- 31 My patient is on a proton pump inhibitor. Does that not affect absorption?

OTHER – 6

- 32 BRUKINSA was third to market. Why would I move now?
- 33 Trial populations are not my patients. What does the real-world evidence show?
- 34 You keep talking about CLL. What about my Waldenström's patients?
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Fixed duration

BACK TO CONTENTS

OBJECTION 1 · FIXED DURATION

“Acalabrutinib plus venetoclax is an appropriate option for my treatment-naive patients — it has good efficacy and it is fixed duration.”

RESPONSE

Acknowledge: “Absolutely, fixed-duration combinations are an important option, and venetoclax-based regimens have strong efficacy.”

“In the same time it is critical to anchor treatment choice on the strongest frontline BTKi evidence, especially for long-term disease control and diverse patient profiles.”

SEQUOIA Arm A – Frontline CLL: Zanubrutinib demonstrated superior PFS vs BR. Effective in TN patients with and without del(17p). Confirms Zanubrutinib as a robust frontline standard.

SEQUOIA Arm C – High-Risk CLL: 6-year durable disease control in del(17p)/TP53-mutated patients. This is the hardest-to-treat population, where depth and durability matter most.

KEY MESSAGE

Zanubrutinib delivers proven long-term control, not just short-term response.

SUPPORTING DATA POINTS

- AMPLIFY was studied in a highly preselected patient population that excluded high-risk features and is not representative of the real-world CLL patient population.
- Median age of patients in the AMPLIFY trial was 61 years (versus the median age at CLL diagnosis is 72 years old).
- 73.2% of patients in the AMPLIFY trial were ≤65 years old.
- All patients in the AMPLIFY trial had a low comorbidity burden: All patients had a CIRS-Geriatric score of ≤6 and 91.8% had an ECOG score of 0 or 1.
- Excluded patients with del(17p)/TP53 mutations (account for ~15% of all patients).
- 36-month PFS: AV 76.5%; AVO 83.1%; FCR/BR 66.5%.
- CR/CRi: AV 8.9%; AVO 15.7%; FCR/BR 5.5%.
- uMRD: AV 26.8%; AVO 66.4%; FCR/BR 51.0%.
- AMPLIFY excluded patients with del(17p)/TP53, therefore there is no evidence to suggest AV or AVO are efficacious in this population.

CITATION

Trial: AMPLIFY; SEQUOIA
File: Objection hadler.pdf
Location: slides 2-3 (identical content in Objection hadler.pptx, slides 2-3)

Source date – File carries no date or approval code. Last modified January 2026; cites AMPLIFY (Brown, NEJM 2025).

“Fixed-duration regimens are easier and more convenient for my practice and my patients.”

RESPONSE

Acknowledge: “I understand the appeal of fixed-duration therapy — shorter treatment time sounds attractive for both physicians and patients.”

Clinical Reality – Fixed Duration Is NOT Simpler in Practice. Fixed-duration venetoclax-based regimens require: complex dose ramp-up; strict TLS risk assessment; frequent hospital visits and lab monitoring; inpatient admission in some patients.

Zanubrutinib = True Simplicity. With Zanubrutinib: no ramp-up; no TLS risk; oral, continuous therapy; minimal monitoring; excellent tolerability profile; low discontinuation rates.

This is real-life convenience for both the physician and the patient.

KEY MESSAGE

Zanubrutinib simplifies treatment without compromising outcomes.

SUPPORTING DATA POINTS

- The AMPLIFY study demonstrated that AV did not achieve efficacy outcomes on par with currently available therapies without the addition of obinutuzumab.
- 36-month PFS: AV 76.5%; AVO 83.1%; FCR/BR 66.5%.
- CR/CRi: AV 8.9%; AVO 15.7%; FCR/BR 5.5%.
- uMRD: AV 26.8%; AVO 66.4%; FCR/BR 51.0%.
- SEQUOIA Arm A – TN CLL: PFS rate at 72 months was higher with zanubrutinib vs BR (74% vs 32%, respectively).
- SEQUOIA Arm C – High-Risk: at a median follow-up of 76.7 months in Arm C, the 72-month PFS rate with zanubrutinib was 64%.

CITATION

Trial: AMPLIFY; SEQUOIA Arms A and C

File: Objection hadler.pdf

Location: slides 4–5 (identical content in Objection hadler.pptx, slides 4–5)

Source date – File carries no date or approval code. Last modified January 2026.

“BRUKINSA as a continuous therapy is not as appealing as fixed-duration.”

RESPONSE

Acknowledge: “I understand that you have two main potential options when treating CLL — either BTK inhibitors or fixed-duration therapy, and you have to weigh them up.”

Probe / clarify: In which patient types do you prefer to use fixed-duration and which do you prefer for BTK inhibitor therapy? Which fixed-duration therapy do you prefer to use for these patients? What do you see as the relative merits for both fixed-duration and continuous therapies? What patient lifestyle factors do you take into account when deciding whether to use continuous or fixed duration therapy?

Respond: Efficacy and Safety of Zanubrutinib Versus Venetoclax+Ibrutinib in Treatment-Naïve Chronic Lymphocytic Leukemia: A Matching-Adjusted Indirect Comparison. BRUKINSA has proven and consistent efficacy regardless of mutations or patient features. It demonstrated its activity across widest range of indications. The only BTKi showing superiority vs Ibrutinib. BRUKINSA has a favourable safety profile in CLL “Mayo clinic safety NMA”. You are losing one valid line of treatment for the patient without adding to the current efficacy ... treatment sequencing.

SUPPORTING DATA POINTS

- No numerical data are given on the response slide itself; the supporting PFS table sits on slide 8 of the same file — see the card “Ibrutinib + venetoclax is the best option for my young patients”.

CITATION

Trial: SEQUOIA; ALPINE; GLOW; CAPTIVATE
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slides 5-7 (Objection #1)

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“Ibrutinib plus venetoclax is the best option for my young patients who want deep remission and time off treatment. We have strong data from GLOW and CAPTIVATE.”

RESPONSE

Acknowledge: “You’re absolutely right — GLOW and CAPTIVATE showed that Ibrutinib + Venetoclax can achieve deep remissions and MRD negativity, especially in younger patients. However, being young does not automatically mean a patient needs intensive combination therapy.”

Clinical Reality – Overtreatment Risk in Young Patients: Many young CLL patients have favorable biology and indolent disease course; can achieve long-term disease control with BTKi monotherapy. Using combination therapy upfront may expose them to unnecessary toxicity without added survival benefit.

BRUKINSA is the only BTKi with superior and sustained PFS vs ibrutinib in a H2H trial in R/R CLL and demonstrates superior PFS vs BR in TN patients. This clinical benefit is consistent across patient subgroups, regardless of age, mutation status, including unmutated IGHV or genetic risk factors, such as del(17p) or TP53 mutations, providing confidence in its broad and durable efficacy.

BRUKINSA provides unmatched dosing flexibility. It’s the only BTKi that allows both once-daily and twice-daily dosing at home with no complicated ramp up or need for TLS monitoring, giving you the ability to tailor treatment schedules to each patient’s lifestyle and medical needs.

SUPPORTING DATA POINTS

- GLOW, Ven+I (n=106) vs Clb+O (n=105), 46.0 months mFU — All treated patients: 42-month PFS: 74.6% vs 24.8% (HR: 0.214; 95% CI, 0.138, 0.334; p<0.0001).
- CAPTIVATE, fixed dose Ven+I (n=159, single-arm cohort with no comparator), 49.8 months mFU — All treated patients: 4-year PFS: 79% (95% CI: 71, 84); IGHV unmutated: 4-year PFS: 73% (95% CI: 62, 81); Del(17p)/TP53 mutated: 4-year PFS: 63% (95% CI: 41, 79).
- SEQUOIA Cohort 1 (patients without del[17p]), BRUKINSA (n=241) vs BR (n=238), 43.7 months mFU — All cohort 1 42-month PFS: 82.4% vs 50.0% (HR: 0.30; 95% CI: 0.21, 0.43; p<0.0001); IGHV mutated (HR: 0.35; 95% CI: 0.19, 0.64; p=0.00033); IGHV unmutated (HR: 0.23; 95% CI: 0.14, 0.37; p<0.0001).
- SEQUOIA Cohort 2 (del[17p] patients only, single-arm, n=111), 47.9 months mFU — All cohort 2 42-month PFS: 79.4%.
- ALPINE, BRUKINSA (n=327) vs ibrutinib (n=325), 39.0 months mFU — All treated patients: 36-month PFS: 64.9% vs 54.8% (HR: 0.68; 95% CI: 0.53, 0.86; 2-sided descriptive p=0.0011).
- The approved slide states plainly: “No head-to-head comparison data is available.”

CITATION

Trial: GLOW; CAPTIVATE (response)
File: Objection hadler.pdf
Location: slides 6–7 (Objection 3)

Trial: SEQUOIA; ALPINE; GLOW; CAPTIVATE (PFS table)
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 8, headed “Objection #4: Prefer Ven+I fixed-duration therapy (supporting data)”

Source date – Response: undated file, last modified January 2026. Supporting PFS table: October 2024.

“AV gives durable PFS without keeping the patient on long-term treatment.”

RESPONSE

BeOne lens: The 36-month PFS rate for AV (76.5%) is similar to that achieved over 72 months with BRUKINSA (74%); with nearly one-quarter of patients progressing by 36 months, this high rate of progression casts doubt on the suitability of AV as a long-term treatment option.

AV does not fulfill the potential of a ‘fixed-duration regimen’ as the response is neither deep nor durable, raising concerns around the risk for rapid relapse. Nearly a quarter of patients had relapsed or died after 36 months. AV only achieved a uMRD rate of 26.8% and a CR rate of 8.9%.

No significant OS benefit vs. FCR/BR (after adjusting for multiplicity) and offers no compelling evidence of superiority versus current SOC options, such as BTKi monotherapy.

HCP prompts: What concerns do you have about the risk of relapse when you stop fixed-duration therapy? What is the treatment goal in 1L CLL? What length of clinical follow-up do you think is appropriate to properly assess the efficacy profile of a treatment in CLL?

KEY MESSAGE

BRUKINSA, as an oral monotherapy, delivers durable efficacy, sustained over long-term follow-up in 1L CLL/SLL.

SUPPORTING DATA POINTS

- AV proof points as stated by AZ: AV significantly prolonged PFS vs. FCR/BR after 40.8 months of follow-up; 36-month PFS: 76.5% vs. 66.5%. ORR rates were 92.8% with AV vs. 75.2% with FCR/BR. AV significantly prolonged OS vs. FCR/BR.
- SEQUOIA primary endpoint PFS: 58% reduced risk vs. BR (HR: 0.42; 95%: 0.28, 0.63; $p < 0.0001$) at 26.2-month mFU.
- SEQUOIA extended ~6-year analysis: 72% reduced risk vs. BR at 72-month mFU (HR: 0.28; 95% CI: 0.20, 0.38; $p < 0.0001$).
- Patients with del(17p) at 6 years in 1L CLL: 72-month PFS rate of 64% (95% CI: 53.6, 72.8).
- NMA of RCTs in low-risk 1L CLL: 59% reduced risk of progression or death with BRUKINSA vs. AV (HR: 0.41; 95% CrI: 0.25, 0.67); 3-year PFS: 85.6% with BRUKINSA vs. 76.5% with AV.
- MAIC of fit patients in 1L CLL: 55% reduced risk of progression or death with BRUKINSA vs. AV (HR: 0.45; 95% CI: 0.23, 0.88; $p = 0.0197$); 3-year PFS: 89.2% with BRUKINSA vs. 78.9% with AV.

CITATION

Trial: AMPLIFY; SEQUOIA; anchored MAIC SEQUOIA vs AMPLIFY (Shadman, Blood Adv 2026); NMA (Shadman, ASCO 2025 e19031)

File: Objection Handler AMPLIFY.pptx

Location: slide 13 (AZ Claim 1)

Source date – April 2026 (1225-BRU-PRC-030).

“AVO is a fixed-duration treatment that drives a deep response in 1L CLL with a manageable safety profile.”

RESPONSE

BeOne lens: AVO showed no statistically significant OS benefit vs FCR/BR, and offers no compelling evidence of superiority versus current SOC options, such as BTKi monotherapy (BRUKINSA 42-month PFS: 82.4%).

Even with the combined triplet therapy, one-third of patients did not achieve uMRD. Adding obinutuzumab increases safety burden without providing a survival advantage.

US PIs also included cautionary language on use of AVO in Warnings & Precautions section: Serious and Opportunistic Infections (acalabrutinib US PI) / Infections (venetoclax US PI). 23.6% of patients had grade ≥ 3 infections and 6% had a serious AE leading to death.

AVO demonstrated a high risk of cardiac disorders, even in a 'fitter' population than would be expected in real-world practice.

HCP prompts: What safety considerations do you take into account when using three products? What level of efficacy are you hoping for by using three products?

KEY MESSAGE

BRUKINSA has demonstrated consistent efficacy across all patient types enrolled in SEQUOIA and has shown a favorable and consistent safety profile over time, with stable or decreasing AEs with long-term follow-up and no new safety signals.

SUPPORTING DATA POINTS

- AVO proof points as stated by AZ: AVO significantly prolonged PFS vs. FCR/BR after 40.8 months of follow-up; 36-month PFS: 83.1% vs. 66.5%. ORR rates were 92.7% with AVO vs. 75.2% with FCR/BR. uMRD rates were 66.4% with AVO vs. 51.0% with FCR/BR.
- 83.6% of patients on BRUKINSA monotherapy remain progression free at 3 years.
- BRUKINSA prolonged PFS vs. BR in fit patients (mFU: 40.3 months): PFS at 42 months: 87.1% with BRUKINSA vs. 50.0% with BR; 77% reduced risk of disease progression or death (HR: 0.23; 95% CI: 0.13, 0.40; $p < 0.0001$). Exploratory post hoc analysis ($n=252$) in Cohort 1 excluding patients with SLL, del(17p), TP53 mutation, baseline creatinine clearance < 50 mL/min, and CIRS score > 6 .
- Cohort 2 is the largest prospective study to date of patients with TN CLL and del(17p) ($n=110$); 6-year PFS del(17p): 64% (95% CI: 53.6, 72.8) at 76.7 months mFU.
- Rate of any-grade / grade ≥ 3 atrial fibrillation: 3.3% / 0.4% with BRUKINSA vs. 2.6% / 1.3% with BR (26.2-month mFU).
- Rate of any-grade / grade ≥ 3 hypertension: 14.2% / 6.3% with BRUKINSA vs. 10.6% / 4.8% with BR (26.2-month mFU).

CITATION

Trial: AMPLIFY; SEQUOIA

File: Objection Handler AMPLIFY.pptx

Location: AZ Claim 6 slide – immediately follows slide 18; the slide number itself was not captured in the text layer

OBJECTION 7 · FIXED DURATION

“Fixed duration is cheaper. A finite course costs the system less than years of continuous therapy.”

RESPONSE

Refer this question to Market Access. Pricing, tender and budget positions are set locally and are not covered by the approved clinical materials.

What you can speak to is the clinical trade-off rather than the cost one: with a fixed-duration regimen you are using up one valid line of treatment for the patient without adding to the current efficacy — a treatment-sequencing consideration.

SUPPORTING DATA POINTS

- The clinical point available in the approved materials is a sequencing one: “You are loosing one valid line of treatment for the patient without adding to the current efficacy ... treatment sequencing.”

CITATION

Trial: –
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 7

Efficacy

BACK TO CONTENTS

OBJECTION 8 · EFFICACY

“Show me the long-term frontline data. How long does the benefit actually last?”

RESPONSE

Superior PFS vs. BR in 1L CLL at the primary endpoint analysis supported by the extended analysis at ~6-year mFU.

Proven PFS benefit with BRUKINSA regardless of mutation or risk status.

KEY MESSAGE

BRUKINSA, as an oral monotherapy, delivers durable efficacy, sustained over long-term follow-up in 1L CLL/SLL.

SUPPORTING DATA POINTS

- Primary endpoint PFS: 58% reduced risk vs. BR (HR: 0.42; 95%: 0.28, 0.63; p<0.0001) at 26.2-month mFU.
- Extended ~6-year analysis: 72% reduced risk vs. BR at 72-month mFU (HR: 0.28; 95% CI: 0.20, 0.38; p<0.0001).
- 6-year PFS non-del(17p): 74% (95% CI: 67.2, 79.1) with BRUKINSA vs. 32% (95% CI: 25.2, 39.9) with BR at a mFU of 72.8 months.
- Patients with uIGHV: 78% reduced risk of progression vs BR; HR: 0.22; p<0.0001. Similar PFS between mIGHV and uIGHV in BRUKINSA-treated patients.
- SEQUOIA Cohort 1 patients treated with BRUKINSA: median age 70 years (81% aged ≥65 years); 54% had an ECOG PS score of 1 or 2.
- From the 5-year follow-up of Cohort 1: estimated 60-month PFS rates were 75.8% and 40.1% in zanubrutinib- and BR-treated patients, respectively. With adjustment for COVID-19 impact, the respective 60-month PFS rates were 78.7% and 40.6%. (Data cutoff 30 April 2024.)

CITATION

Trial: SEQUOIA (Tam, ASH 2025 Poster 2129; Tam, Lancet Oncol 2022)
File: Objection Handler AMPLIFY.pptx
Location: slides 13 and 15

Trial: SEQUOIA 5-year follow-up (Shadman, J Clin Oncol 2024/2025)
File: ZANU ALL INDICATIONS.pdf
Location: section “SEQUOIA – 5-year follow-up” (this file prints no slide numbers)

Source date – April 2026 for the 6-year figures; SEQUOIA 5-year data cutoff 30 April 2024.

“What have you got in del(17p) or TP53-mutated patients? That is where I actually struggle.”

RESPONSE

Patients with traditionally poor prognostic factors, such as del(17p) mutation or uIGHV disease, continue to experience durable remission in the 6-year follow-up analysis of SEQUOIA.

Cohort 2 is the largest prospective study to date of patients with TN CLL and del(17p) (n=110).

From the SEQUOIA Arm C 5-year author conclusions: the estimated 60-month PFS with zanubrutinib was 72.2%, similar to that observed in patients without del(17p), highlighting that zanubrutinib overcomes the negative prognostic impact of del(17p). Zanubrutinib remains a valuable frontline treatment option for patients with CLL/SLL with or without del(17p).

KEY MESSAGE

BRUKINSA has been studied in a diverse patient population, reflective of real-world patients, including patients with poor prognostic features.

SUPPORTING DATA POINTS

- 6-year PFS del(17p): 64% (95% CI: 53.6, 72.8) with BRUKINSA at a mFU of 76.7 months.
- SEQUOIA Cohort 2 patients: median age 70 years (86% aged ≥65 years); 60% had an ECOG PS score of 1 or 2; 43% had TP53 mutation.
- SEQUOIA Arm C 5-year follow-up (data cutoff 30 April 2024, n=110): ORR 107 (97.3%); CR/CRi 20 (18.2%); nPR 3 (2.7%); PR 84 (76.4%); SD 2 (1.8%); PD 1 (0.9%). Estimated 60-month PFS 72.2%.
- PFS benefit at 6 years was similar between patients with del(17p) and both mIGHV and uIGHV treated with BRUKINSA (HR: 1.21; p=0.0585).
- ALPINE, R/R setting — Del(17p)/TP53 mutated: 36-month PFS 58.6% vs 41.3% with ibrutinib (HR: 0.52; 95% CI: 0.33, 0.83; 2-sided descriptive p=0.0047).
- AMPLIFY excluded patients with del(17p) or TP53 mutation, which accounts for ~15% of newly diagnosed patients.

CITATION

Trial: SEQUOIA Cohort 2 (Tam, ASH 2025 Poster 2129)

File: Objection Handler AMPLIFY.pptx

Location: slides 15 and 16

Trial: SEQUOIA Arm C 5-year (Tam, ASCO 2025 abstract 7011)

File: ZANU ALL INDICATIONS.pdf

Location: section “SEQUOIA Arm C: 5-year follow-up” (this file prints no slide numbers)

Trial: ALPINE

File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf

Location: slide 8

Source date – April 2026 for the 6-year figures; Arm C 5-year data cutoff 30 April 2024; ALPINE table October 2024.

“The AMPLIFY results apply to the patients I see. Why would I not generalise them?”

RESPONSE

BeOne lens: Because AMPLIFY’s patient population does not reflect real-world patients with CLL, its outcomes are not generalizable.

The median age of patients enrolled in AMPLIFY was 61 years, but the median age of CLL diagnosis is 70 years, with treatment typically initiating between the ages of ~72 and 78 years.

73.2% of patients enrolled in AMPLIFY were aged <65 years, but only 31% of newly diagnosed CLL patients are aged <65 years.

Patients with del(17p) or TP53 mutation were excluded, which accounts for ~15% of newly diagnosed patients. Patients aged ≥65 years were excluded if they had a total score of >6 on the Cumulative Illness Rating Scale for Geriatrics.

HCP prompts: How do you approach choice of therapy for patients with high-risk features? When trials more accurately mirror your clinical practice, how does that impact your interpretation of the results?

KEY MESSAGE

BRUKINSA has been studied in a diverse patient population, reflective of real-world patients, including patients with poor prognostic features.

SUPPORTING DATA POINTS

- AMPLIFY is a Phase 3, multicenter, open-label, randomized trial of adults aged ≥18 years with previously untreated CLL, ECOG performance-status score of 0–2 who did not have del(17p) or TP53 mutation.
- SEQUOIA enrolled a broad patient population that reflected the diversity of real-world populations. Cohort 1 (without del[17p]) treated with BRUKINSA: median age 70 years (81% aged ≥65 years); 54% had an ECOG PS score of 1 or 2.
- Cohort 2 (with del[17p]): median age 70 years (86% aged ≥65 years); 60% had an ECOG PS score of 1 or 2; 43% had TP53 mutation.
- Nearly three-quarters of patients (74%) remain progression free after 6 years of follow-up: 6-year PFS non-del(17p) 74% (95% CI: 67.2, 79.1) with BRUKINSA vs. 32% (95% CI: 25.2, 39.9) with BR at a mFU of 72.8 months.

CITATION

Trial: AMPLIFY (Brown, NEJM 2025); SEQUOIA (Tam, Lancet Oncol 2022; Tam, ASH 2025 Poster 2129)
File: Objection Handler AMPLIFY.pptx
Location: slide 15 (AZ Claim 3)

Source date – April 2026 (1225-BRU-PRC-030).

“PFS improved with AV versus chemotherapy regardless of IGHV status, so unmutated IGHV is no longer a reason to pick a BTK inhibitor.”

RESPONSE

BeOne lens: In a highly preselected population of young, fit patients (median age: 61 years; patients with del(17p) or TP53 mutation excluded from the study), the PFS data for AV are underwhelming, including for patients with mIGHV and uIGHV.

A median follow-up of 40.8 months is insufficient to ensure patients may safely cease treatment and provide confidence in sustained PFS.

HCP prompts: What level of efficacy would you want to see when considering a 1L CLL treatment? For how long would you need efficacy to be demonstrated before you feel confident using a treatment? How confident are you that fixed-duration therapy provides adequate control in patients with historically poor prognosis?

KEY MESSAGE

BRUKINSA elevates the foundation of care for 1L CLL, with superior and enduring outcomes over BR across patient profiles.

SUPPORTING DATA POINTS

- AV proof point as stated by AZ: The treatment effect on PFS of AV vs. FCR/BR was similar across the prespecified subgroups, including the subgroup defined according to IGHV status (median follow-up: 40.8 months). HR for disease progression or death (95% CI) with AV vs. FCR/BR: mIGHV, 0.67 (0.39, 1.14); uIGHV, 0.69 (0.48, 0.97).
- SEQUOIA, patients with uIGHV: 78% reduced risk of progression vs. BR; HR: 0.22; $p < 0.0001$. Similar PFS between mIGHV and uIGHV in BRUKINSA-treated patients.
- Number of PFS events at 6 years: BRUKINSA with mIGHV $n=12$ (33%); BRUKINSA with uIGHV $n=25$ (37%).
- Elderly 1L CLL patients at 5 years (exploratory analysis): without del(17p), 5-year PFS 69% vs 44% with BR; with del(17p), 5-year PFS 69%.
- 6-year PFS non-del(17p): 74% (95% CI: 67.2, 79.1) with BRUKINSA vs 32% (95% CI: 25.2, 39.9) with BR at a mFU of 72.8 months.

CITATION

Trial: AMPLIFY (Brown, NEJM 2025); SEQUOIA (Tam, ASH 2025 Poster 2129); Tedeschi, iwCLL 2025 Poster 1421
File: Objection Handler AMPLIFY.pptx
Location: slide 16 (AZ Claim 4)

Source date – April 2026 (1225-BRU-PRC-030).

“AMPLIFY showed an overall survival benefit. Has BRUKINSA shown one?”

RESPONSE

On the AV side there is an approved response: BeOne lens — “No significant OS benefit vs. FCR/BR (after adjusting for multiplicity) and offers no compelling evidence of superiority versus current SOC options, such as BTKi monotherapy.” For AVO: “AVO showed no statistically significant OS benefit vs FCR/BR.”

SUPPORTING DATA POINTS

- SEQUOIA 5-year follow-up (Cohort 1, data cutoff 30 April 2024): Median OS was not reached in either treatment arm (HR, 0.89 [95% CI, 0.55 to 1.43]; descriptive P=0.3090); estimated 60-month OS rates were 85.8% and 85.0% in zanubrutinib- and BR-treated patients, respectively.
- With adjustment for COVID-19 impact, estimated 60-month OS rates were 89.4% and 86.8% in zanubrutinib- and BR-treated patients, respectively.

CITATION

Trial: AMPLIFY (Brown, NEJM 2025)
File: Objection Handler AMPLIFY.pptx
Location: slide 13 (AZ Claim 1) and AZ Claim 6 slide

Trial: SEQUOIA 5-year follow-up (Shadman, J Clin Oncol)
File: ZANU ALL INDICATIONS.pdf
Location: section “SEQUOIA – 5-year follow-up” (this file prints no slide numbers)

Source date – April 2026 for the AV/AVO response; SEQUOIA OS data cutoff 30 April 2024.

“Why would I use BRUKINSA in relapsed disease rather than staying with what I know?”

RESPONSE

BRUKINSA is the only BTKi with superior and sustained PFS vs ibrutinib in a H2H trial in R/R CLL and demonstrates superior PFS vs BR in TN patients. This clinical benefit is consistent across patient subgroups, regardless of age, mutation status, including unmutated IGHV or genetic risk factors, such as del(17p) or TP53 mutations, providing confidence in its broad and durable efficacy.

SUPPORTING DATA POINTS

- ALPINE, BRUKINSA (n=327) vs ibrutinib (n=325), 39.0 months mFU — All treated patients: 36-month PFS 64.9% vs 54.8% (HR: 0.68; 95% CI: 0.53, 0.86; 2-sided descriptive p=0.0011).
- ALPINE — Del(17p)/TP53 mutated: 36-month PFS 58.6% vs 41.3% (HR: 0.52; 95% CI: 0.33, 0.83; 2-sided descriptive p=0.0047).
- ALPINE interim analysis, primary endpoint investigator-assessed ORR (PR+CR): 162 (78.3%) with zanubrutinib vs 130 (62.5%) with ibrutinib; superiority 2-sided P=0.0006 compared with pre-specified alpha of 0.0099.
- ALPINE, del(17p) subgroup at interim: ORR (PR+CR) 20 (83.3%) of n=24 with zanubrutinib vs 14 (53.8%) of n=26 with ibrutinib.
- ALPINE final comparative analysis (data cut-off 28 Feb 2024), median study follow-up 42.5 months: median OS had not been reached in either treatment group.

CITATION

Trial: ALPINE (response text)
File: Objection hadler.pdf
Location: slide 6

Trial: ALPINE 36-month PFS
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 8

Trial: ALPINE interim and final comparative analysis (Brown, Blood 2024)
File: ZANU ALL INDICATIONS.pdf
Location: sections “ALPINE” and “ALPINE – Final Comparative Analysis” (this file prints no slide numbers)

Source date – Response from an undated file last modified January 2026; PFS table October 2024; ALPINE final analysis data cut-off 28 Feb 2024.

Safety and tolerability

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“The adverse events with AV were mostly grade 1 or 2, and the rates of the cardiovascular events I care about were low.”

RESPONSE

BeOne lens: AV raises serious safety concerns throughout the 60 weeks of treatment.

Comparable rates of serious AEs (24.7% vs. 27.4%), AEs leading to deaths (3.4% vs. 3.5%) and Grade ≥ 3 infections (12.4% vs. 10.0%) compared with CIT.

High rates of discontinuations and dose reductions due to AEs (7.6% and 5.8%, respectively).

AV demonstrated a high risk of cardiac disorders, even in a ‘fitter’ population than would be expected in real-world practice.

HCP prompts: Which AEs concern you and patients the most when treating 1L CLL? What AE rates do you expect for a chemo-free 1L CLL treatment? To what extent does having substantially longer follow-up data increase your confidence in a therapy’s safety profile?

KEY MESSAGE

BRUKINSA delivers proven, long-term safety and tolerability — empowering physicians to treat with confidence and achieve positive outcomes.

SUPPORTING DATA POINTS

- AV proof points as stated by AZ: Grade 3 AEs were reported in 53.6% of patients in the AV arm. The most common grade 3/4 AE was neutropenia, which was generally manageable. Low rates of any-grade cardiovascular AEs: 0.7% atrial fibrillation/flutter, 4.1% hypertension, 1% major hemorrhage.
- Of the 1,550 patients treated with BRUKINSA across five B-cell malignancies, 4.8% discontinued treatment due to AEs and 5.0% underwent dose reductions.
- BRUKINSA in 1L CLL at 26.2-month mFU — Atrial fibrillation, any grade: 3.3% with BRUKINSA vs. 2.6% with BR; Grade ≥ 3 : 0.4% with BRUKINSA vs. 1.3% with BR.
- BRUKINSA in 1L CLL at 26.2-month mFU — Hypertension, any grade: 14.2% with BRUKINSA vs. 10.6% with BR; Grade ≥ 3 : 6.3% with BRUKINSA vs. 4.8% with BR.
- Retrospective real-world study (TriNetX) in 1L CLL: 51% lower risk of new-onset atrial fibrillation/flutter vs. acalabrutinib (RR: 0.49, 95% CI: 0.27, 0.91; $p=0.020$).
- Same study: 27% lower risk of major cardiovascular AEs vs. acalabrutinib (RR: 0.73; 95% CI: 0.58, 0.93; $p=0.009$). Risks of new-onset hypertension, arrhythmia, new-onset heart failure, myocardial infarction, conduction block, and stroke were similar between groups.

CITATION

Trial: AMPLIFY (Brown, NEJM 2025); SEQUOIA (Tam, Lancet Oncol 2022); TriNetX (Tang, Blood 2024)
File: Objection Handler AMPLIFY.pptx
Location: slide 14 (AZ Claim 2)

Source date — April 2026 (1225-BRU-PRC-030).

“I am worried about atrial fibrillation. All the BTK inhibitors cause it.”**RESPONSE**

Approved message from the field guidance: BRUKINSA delivers proven, long-term safety and tolerability — empowering physicians to treat with confidence and achieve positive outcomes.

From the pooled safety analysis conclusions: this relatively stable incidence of afib/flutter with zanubrutinib, despite extended exposure, is important for long-term treatment. Additionally, a lower incidence of afib may minimize the need for supportive care (eg, anticoagulants, antiplatelet agents) that can further increase the bleeding risk associated with BTKis.

SUPPORTING DATA POINTS

- SEQUOIA, 1L CLL, 26.2-month mFU — Atrial fibrillation any grade: 3.3% with BRUKINSA vs. 2.6% with BR. Grade ≥ 3 : 0.4% vs. 1.3%.
- Pooled zanubrutinib monotherapy (n=779): Atrial fibrillation or flutter was reported in 3% of patients. Grade ≥ 3 atrial fibrillation/flutter was reported in 6 patients (1%). No events led to treatment discontinuation or death.
- Pooled (n=779): All grade atrial fibrillation/flutter occurred at a rate of 0.1 events per 100 person-months; grade ≥ 3 at a rate of 0.03 events per 100 person-months. Median time to onset 183 (36–622) days.
- Pooled (n=779): 8 patients (1%) experienced ventricular tachyarrhythmias; none led to zanubrutinib discontinuation or death.
- ALPINE final comparative analysis (data cut-off 28 Feb 2024), zanubrutinib n=324 vs ibrutinib n=324: Atrial fibrillation/flutter 23 (7.1%) vs 55 (17.0%). Patients with ≥ 1 cardiac TEAE 84 (25.9%) vs 115 (35.5%). Cardiac AEs leading to treatment discontinuation 3 (0.9%) vs 16 (4.9%). Fatal cardiac AEs 0 vs 6 (1.9%).
- Brown, Haematologica 2024 comparative analysis (ASPEN cohort 1 + ALPINE): the EAIR for afib/flutter was significantly lower with zanubrutinib vs ibrutinib ($P < .0001$). At all treatment intervals, the EAIR was 6.7 to 13.6 persons per 100 PY higher with ibrutinib vs zanubrutinib.

CITATION

Trial: SEQUOIA; TriNetX
File: Objection Handler AMPLIFY.pptx
Location: slide 14

Trial: Pooled zanubrutinib monotherapy (Tam, Blood Adv 2022)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 20 and 22

Trial: ASPEN + ALPINE (Brown, Haematologica 2024)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 63 and 68

Trial: ALPINE final comparative analysis (Brown, Blood 2024)
File: ZANU ALL INDICATIONS.pdf
Location: section “ALPINE – Final Comparative Analysis: Summary of Treatment-Emergent and Cardiac Adverse Events” (this file prints no slide numbers)

Source date – Newest element April 2026; pooled safety data May 2025; ALPINE final analysis data cut-off 28 Feb 2024.

“What about hypertension? That is the adverse event I actually see in clinic.”**RESPONSE**

BRUKINSA delivers proven, long-term safety and tolerability — empowering physicians to treat with confidence and achieve positive outcomes.

From the pooled safety analysis: across all 10 trials, no zanubrutinib-treated patients discontinued due to hypertension.

In ALPINE, the hypertension rate was similar between the zanubrutinib and ibrutinib arms; however, the incidence of cardiac disorders such as afib/flutter was higher in the ibrutinib arm. Incidence in the zanubrutinib arm remained low and comparable to that observed in SEQUOIA and ASPEN.

Although hypertension in patients receiving ibrutinib has been associated with increased incidence of major cardiovascular AEs, the incidence of cardiac disorder TEAEs was comparable for zanubrutinib across ALPINE, ASPEN, and SEQUOIA despite higher hypertension EAIR observed in ALPINE.

KEY MESSAGE

BRUKINSA delivers proven, long-term safety and tolerability, empowering physicians to treat with confidence and achieve positive outcomes.

SUPPORTING DATA POINTS

- SEQUOIA, 1L CLL, 26.2-month mFU — Hypertension any grade: 14.2% with BRUKINSA vs. 10.6% with BR. Grade ≥ 3 : 6.3% vs. 4.8%.
- Pooled zanubrutinib monotherapy (n=779): All grade hypertension was reported in 12% of patients; grade ≥ 3 hypertension in 5%. All grade rate 0.6 events per 100 person-months; grade ≥ 3 rate 0.2 events per 100 person-months.
- Pooled (n=779): 15% of patients with a history of hypertension developed treatment-emergent hypertension; 11% of patients without a history of hypertension developed treatment-emergent hypertension. Median time to onset of hypertension was 227 days (68–521).
- Brown, Haematologica 2024 (N=1550 across 10 trials): “Importantly, across all 10 trials, no zanubrutinib-treated patients discontinued due to hypertension.”
- Brown, Haematologica 2024: ALPINE had a greater hypertension EAIR than SEQUOIA and ASPEN; exclusion of data from ALPINE decreased the hypertension EAIR to 5.73 persons per 100 PY. In ALPINE, the hypertension rate was similar between the zanubrutinib and ibrutinib arms. The hypertension EAIR was reduced in patients receiving zanubrutinib vs ibrutinib (P=.0610).
- ALPINE final comparative analysis: any-grade hypertension 27.2% with zanubrutinib vs 25.3% with ibrutinib.

CITATION

Trial: SEQUOIA
File: Objection Handler AMPLIFY.pptx
Location: slide 14

Trial: Pooled zanubrutinib monotherapy (Tam, Blood Adv 2022)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 19 and 22

Trial: Brown, Haematologica 2024

File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx

Location: slide 62

Trial: ALPINE final comparative analysis (Brown, Blood 2024)

File: ZANU ALL INDICATIONS.pdf

Location: section "ALPINE – Final Comparative Analysis: All-grade TEAEs Occurring in $\geq 15\%$ of Patients" (no slide numbers printed)

Source date – Pooled safety May 2025; SEQUOIA figures via April 2026 deck; ALPINE final analysis data cut-off 28 Feb 2024.

“If the patient is on it for years, they will eventually come off it for toxicity anyway.”

RESPONSE

From the pooled safety analysis conclusions: in the comparative analysis, zanubrutinib exhibited a more favorable safety profile than ibrutinib, as demonstrated by the longer median treatment duration and lower frequency of TEAEs, including cardiac disorders, that led to treatment discontinuation or death.

These analyses support zanubrutinib as an appropriate long-term treatment option for patients with B-cell malignancies.

In this analysis, zanubrutinib remained generally well tolerated, consistent with the previous analysis, with no emergence of new safety signals, even at a median treatment duration of approximately 3 years.

SUPPORTING DATA POINTS

- Of the 1,550 patients treated with BRUKINSA across five B-cell malignancies, 4.8% discontinued treatment due to AEs and 5.0% underwent dose reductions (BRUKINSA EU SmPC, November 2025).
- Brown, Haematologica 2024, all zanubrutinib (N=1550): TEAEs leading to treatment discontinuation 211 (13.6%); leading to dose reduction 156 (10.1%); leading to dose interruption 791 (51.0%).
- Comparative analysis (ASPEN cohort 1 + ALPINE): TEAEs leading to treatment discontinuation, zanubrutinib 60 (14.1%) vs ibrutinib 93 (22.0%). Cardiac disorder TEAEs leading to discontinuation 2 (0.5%) vs 18 (4.3%).
- Comparative analysis: median treatment duration 32.6 months (range 0.4–68.7) for zanubrutinib vs 25.7 months (range 0.1–59.3) for ibrutinib. Median time to discontinuation by Kaplan-Meier estimate 63.3 vs 42.2 months.
- Comparative analysis: zanubrutinib-treated patients were more likely to still be on treatment at data cutoff vs ibrutinib (69.9% vs 45.0%).
- In the total pooled zanubrutinib population, 45.0% of patients received zanubrutinib for ≥36 months (median 34.4 months, range 0.1–90.0) and 56.5% remained on zanubrutinib as of the data cutoff.

CITATION

Trial: BRUKINSA EU SmPC November 2025
File: Objection Handler AMPLIFY.pptx
Location: slide 14

Trial: Pooled zanubrutinib N=1550; ASPEN cohort 1 + ALPINE (Brown, Haematologica 2024)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 57–59 and 68

Source date – April 2026 (SmPC figure) and May 2025 (pooled safety).

“Infections are what actually kill my CLL patients. What does BRUKINSA do to infection risk?”

RESPONSE

BRUKINSA delivers proven, long-term safety and tolerability — empowering physicians to treat with confidence and achieve positive outcomes.

From the pooled safety analysis: infections were the most frequently reported category of AEs. The majority were grade 1–2 in maximum severity and typically occurred early in treatment.

In the comparative analysis, the infection EAIR was significantly lower with zanubrutinib than ibrutinib (64.81 vs 79.63 persons per 100 PY; $P=.0098$), even after excluding COVID-19–related infection terms (54.48 vs 69.96 persons per 100 PY; $P=.0029$).

These analyses support zanubrutinib as an appropriate long-term treatment option for patients with B-cell malignancies.

KEY MESSAGE

Zanubrutinib was generally well tolerated; AEs could be monitored with conventional safety assessments, were manageable, and mostly reversible.

SUPPORTING DATA POINTS

- Pooled zanubrutinib monotherapy (n=779): Infections were the most frequently reported category of AEs. The majority were grade 1–2 in maximum severity and typically occurred early in treatment.
- Pooled (n=779): Infections leading to death occurred in 2% of patients; infections leading to treatment discontinuation occurred in 3% of patients. Pneumonia was the most common grade ≥ 3 nonhematologic AE (11%).
- Brown, Haematologica 2024, all zanubrutinib (N=1550): TEAEs leading to death 113 (7.3%), of which infections and infestations 57 (3.7%). Pneumonia (8.2%) was the only serious TEAE in $\geq 5\%$ of patients.
- Comparative analysis (ASPEN cohort 1 + ALPINE): the infection EAIR was significantly lower with zanubrutinib than ibrutinib (64.81 vs 79.63 persons per 100 PY; $P=.0098$), even after excluding COVID-19–related infection terms (54.48 vs 69.96 persons per 100 PY; $P=.0029$).
- COVID-19 (including COVID-19 and COVID-19 pneumonia) was the leading cause of death related to infection: total zanubrutinib population n=26; ASPEN/ALPINE zanubrutinib n=13; ASPEN/ALPINE ibrutinib n=15.

CITATION

Trial: Pooled zanubrutinib monotherapy (Tam, Blood Adv 2022)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 19 and 22

Trial: Pooled N=1550 and ASPEN+ALPINE (Brown, Haematologica 2024)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 59, 60 and 63

Source date – May 2025.

“BRUKINSA causes more neutropenia than the alternatives.”**RESPONSE**

From the pooled safety analysis: All EAIRs of AESIs, except for neutropenia, were numerically lower in patients treated with zanubrutinib vs ibrutinib. Although the neutropenia EAIR was slightly higher with zanubrutinib, the infection EAIR was significantly lower (64.81 vs 79.63 persons per 100 PY; P=.0098) with zanubrutinib, even after excluding COVID-19–related infection terms (54.48 vs 69.96 persons per 100 PY; P=.0029).

In the comparative analysis, AESI EAIRs over time were numerically lower with zanubrutinib vs ibrutinib, except for neutropenia, which was higher in the first 12 months of treatment and is considered an on-target effect of BTK inhibition. However, this was not accompanied by elevated infection.

SUPPORTING DATA POINTS

- Pooled zanubrutinib monotherapy (n=779): Cytopenias were relatively frequently reported, usually not serious, and typically managed with supportive care and/or treatment interruption. 5 patients (<1%) discontinued treatment due to cytopenias.
- Brown, Haematologica 2024, all zanubrutinib (N=1550): neutropenia leading to treatment discontinuation, 3 patients (0.2%).

CITATION

Trial: Pooled zanubrutinib; ASPEN cohort 1 + ALPINE (Brown, Haematologica 2024)
File: Zanubrutinib Pooled Safety Data - MA Slides_MRC-approved May 2025.pptx
Location: slides 22, 61, 63 and 64

Source date – May 2025.

“My patient had to stop ibrutinib because of toxicity. Why would BRUKINSA be any different?”

RESPONSE

BGB-3111-215 was designed for exactly this patient: previously treated patients who were intolerant of a prior BTK inhibitor were switched to zanubrutinib.

No recurrent ibrutinib intolerance event resulted in treatment discontinuation.

83% of intolerance events (15/18) did not recur. None recurred at a higher severity — 1 event recurred at a lower severity and 2 recurred at the same severity. Most (7 of 10, 70%) acalabrutinib-treated patients did not have a recurrence of their acalabrutinib intolerance event.

Among 64 efficacy-evaluable patients, 60 (93.8%) had disease control and 41 (64.1%) had an overall response. Note this study enrolled CLL/SLL, WM, MCL and MZL together, so these figures are the mixed-histology population rather than CLL alone.

KEY MESSAGE

Intolerance to one BTK inhibitor does not predict intolerance to zanubrutinib.

SUPPORTING DATA POINTS

- BGB-3111-215 is a Phase 2, open-label, single-arm trial of zanubrutinib in patients with previously treated B-cell malignancies who were intolerant of previous BTK inhibitors (Shadman, Lancet Haematology 2023;10(1):e35–e45).
- Cohort 1 (ibrutinib intolerance, n=57), cohort 2 (acalabrutinib or acalabrutinib and ibrutinib intolerance, n=7). Median age >70 years (71 and 74). CLL 67% and 50% of each cohort.
- “No recurrent ibrutinib intolerance event resulted in treatment discontinuation.”
- For acalabrutinib intolerance events: 83% of intolerance events (15/18) did not recur; 3 acalabrutinib intolerance events recurred; none recurred at a higher severity; 1 event recurred at a lower severity and 2 recurred at the same severity; 1 of the 4 grade 3 intolerance events recurred but at a lower severity. Most (7 [70%]) acalabrutinib-treated patients did not have a recurrence of their acalabrutinib intolerance event.
- Among 64 efficacy-evaluable patients: 60 (93.8%; 95% CI 84.8–98.3) had disease control; 41 (64.1%; 51.1–75.7) had an overall response. Median DoR not reached. Median PFS not reached. 18-month PFS was 83.8% (95% CI 62.6–93.6). Data cutoff 8 September 2021.

CITATION

Trial: BGB-3111-215 (Shadman, Lancet Haematol 2023)

File: ZANU ALL INDICATIONS.pdf

Location: section “Zanubrutinib BGB-3111-215”, code 1122-BGB-3111-MRC-027 (this file prints no slide numbers)

Source date – Data cutoff September 2021; source file contains ASCO/EHA 2025 content but carries no single approval date.

Comparative and ITC

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OBJECTION 21 · COMPARATIVE AND ITC

“BRUKINSA is not giving me extra clinical benefit over acalabrutinib. They are basically the same drug.”

RESPONSE

Acknowledge: “I understand that you have two 2nd generation BTKis and you are trying to get the best for your patients as they deserve so.”

Probe / clarify: What was your motive to use Acalabrutinib and which form is available? What are the main challenges that your patients face with Acala? What do you see as the relative merits for both Acala and Brukinsa? Which CLL patient you will not consider Acala for?

Respond: In KHAN NMA presented in EHA 2022 Brukinsa showed superiority against both Acala and Ibrutinib in PFS (48% RR of progression or death against Acala). Brukinsa is the only BTKi to show superiority vs Ibrutinib in phase 3 randomized clinical trial “Alpine” while Acala PFS HR was 1. Despite coming third Brukinsa has demonstrated its activity across widest range of indications. Favourable safety profile “Mayo Clinic NMA”. Flexible dosing and not impacted by PPIs “In case of Capsule”.

SUPPORTING DATA POINTS

- No figures other than the 48% relative risk reduction are given on this slide. The named KHAN NMA (EHA 2022) and “Mayo Clinic NMA” are not reproduced in any of the seven source files.

CITATION

Trial: KHAN NMA (EHA 2022); ALPINE; “Mayo Clinic NMA”

File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf

Location: slides 9-11 (Objection #2)

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“There is no head-to-head trial against acalabrutinib, so an indirect comparison does not prove anything.”

RESPONSE

The deck states this openly: “RCTs remain the gold standard for determining efficacy and safety for making clinical treatment decisions.” “There are no head-to-head clinical trials comparing zanubrutinib and acalabrutinib.”

“MAICs have inherent methodology limitations, and results should be interpreted with caution based on cross-trial differences and viewed as hypothesis-generating evidence.”

“Zanubrutinib has demonstrated superior efficacy vs ibrutinib in a head-to-head randomized, clinical trial in patients with R/R CLL/SLL.” “In the ELEVATE-RR trial, acalabrutinib demonstrated non-inferiority to ibrutinib in high-risk patients with CLL with del(17p)/TP53 and del(11q).”

Author conclusions: “This comprehensive MAIC demonstrated a significant PFS and CR advantage, and potentially improved OS for zanubrutinib compared with acalabrutinib. Results were robust across multiple sensitivity analyses. While MAICs provide a scientific basis for evaluating hypotheses with regards to treatment efficacy across trials, the gold standard for evaluating evidence of relative efficacy remains randomized controlled trials.”

SUPPORTING DATA POINTS

- ALPINE vs ASCEND MAIC (Shadman, Ther Adv Med Oncol 2025;17:1-8), ALPINE zanubrutinib N=327, ESS=184.8.
- PFS-INV zanubrutinib vs acalabrutinib — unadjusted population: HR 0.77 (0.55–1.07, P=0.1213); base case adjusted: HR 0.68 (0.46–0.99, P=0.0448).
- OS zanubrutinib vs acalabrutinib — HR 0.6 (0.37–0.97, P=0.0354) and HR 0.6 (0.35–1.02, P=0.0575) across the two main-analysis columns.
- Odds of CR zanubrutinib vs acalabrutinib — OR 2.88 (1.18–7.02, P=0.0198) and OR 2.90 (1.13–7.43, P=0.0270) across the two main-analysis columns.
- The deck also notes a prior MAIC by Kittai et al (Am J Hematol 2023) that “showed similar efficacy for zanubrutinib and acalabrutinib (PFS-INV) and different adverse event profiles”, and states that the efficacy results differ because Kittai et al had “different follow-ups between ALPINE and ASCEND, lack of any adjustment for COVID-19, and incomplete matching variables”.

CITATION

Trial: ALPINE vs ASCEND MAIC (Shadman, Ther Adv Med Oncol 2025); ELEVATE-RR (Byrd, J Clin Oncol 2021)

File: Zanubrutinib Indirect Treatment Comparisons_Global_MRC-approved Aug 2025.pdf

Location: slides 5, 26 and 29

Source date – Footer reads “Updated July 2025”; filename says MRC-approved August 2025. Code 0324-BGB-3111-MRC-046.

“Is there any evidence directly comparing BRUKINSA with acalabrutinib plus venetoclax?”

RESPONSE

There is no head-to-head trial. Two different indirect comparisons exist across the source files and they report different numbers for the same question. Both are given below with their sources; use the April 2026 commercial figures with HCPs and treat the July 2025 medical deck as reactive only.

KEY MESSAGE

BRUKINSA, as an oral monotherapy, delivers durable efficacy, sustained over long-term follow-up in 1L CLL/SLL.

SUPPORTING DATA POINTS

- **APRIL 2026 COMMERCIAL FIELD GUIDANCE** — MAIC of fit patients in 1L CLL: 55% reduced risk of progression or death with BRUKINSA vs. AV (HR: 0.45; 95% CI: 0.23, 0.88; $p=0.0197$); 3-year PFS: 89.2% with BRUKINSA vs. 78.9% with AV. Anchored MAIC between SEQUOIA and AMPLIFY (median follow-up 43.9 and 41.0 months); SEQUOIA individual patient data reweighted to match AMPLIFY and adjusted for sex, ECOG PS, disease stage, del(11q) and IGHV mutation status. (Shadman, Blood Adv 2026.)
- **APRIL 2026** — NMA of RCTs in low-risk 1L CLL: 59% reduced risk of progression or death with BRUKINSA vs. AV (HR: 0.41; 95% CI: 0.25, 0.67); 3-year PFS: 85.6% with BRUKINSA vs. 76.5% with AV. (Shadman, ASCO 2025 abstract e19031.)
- **JULY 2025 MEDICAL ITC DECK** — unadjusted comparison of INV-PFS: HR 0.47 (95% CI: 0.28–0.77; $P=.003$). Population-adjusted INV-PFS base case: HR 0.26 (95% CI: 0.13–0.54; $P=.0003$). (Munir, EHA 2025 PS1581.)
- **JULY 2025** — landmark PFS: “The 36-month PFS rate for zanubrutinib was 85.6% before matching and 88.5% after matching in the base case, compared with 76.5% for AV.”
- **JULY 2025** — sensitivity analyses: IRC-PFS HR 0.23 (0.12–0.48, $P<.0001$); unanchored MAIC without the common FCR/BR control arm assumption HR 0.44 (0.21–0.89, $P=.0220$). Across sensitivity scenarios S1–S13 the HR ranged from 0.15 (0.07–0.34) to 0.73 (0.29–1.84, $P=.4992$).
- **Author conclusions (July 2025):** “This MAIC examined the relative efficacy of zanubrutinib versus acalabrutinib plus the BCL2 inhibitor venetoclax and demonstrated a significant PFS advantage for zanubrutinib over AV regimen. Results should be interpreted with considerations of typical MAIC model assumptions. Future analyses upon trial data maturation are warranted.”

CITATION

Trial: MAIC SEQUOIA vs AMPLIFY (Shadman, Blood Adv 2026); NMA (Shadman, ASCO 2025)
File: Objection Handler AMPLIFY.pptx
Location: slide 13

Trial: Anchored MAIC SEQUOIA vs AMPLIFY (Munir, EHA 2025 PS1581)
File: Zanubrutinib Indirect Treatment Comparisons_Global_MRC-approved Aug 2025.pdf
Location: slides 45–52 (methods 46, KM 49, landmark 50, sensitivity 51, conclusions 52)

Source date – April 2026 and July 2025.

“What about venetoclax plus obinutuzumab? That is the fixed-duration regimen I actually use.”

RESPONSE

Author-stated finding: results suggest a potential treatment benefit favoring zanubrutinib in terms of PFS-INV and OS. Sensitivity analyses exploring the impact of using different sets of matching factors showed consistent results.

The deck's standing caveat applies: these slides are not for any promotional purpose, and indirect treatment comparisons do not establish superiority of one drug over another.

SUPPORTING DATA POINTS

- SEQUOIA vs CLL14 MAIC (Munir, ICML 2025 P209). Zanubrutinib vs venetoclax + obinutuzumab.
- PFS-INV — unadjusted ITT: HR 0.66 (95% CI 0.48–0.89; P=.0077). Base case adjusted: HR 0.66 (95% CI 0.44–0.97; P=.0351).
- OS — unadjusted ITT: HR 0.78 (0.52–1.18; P=.2423). Base case adjusted: HR 0.89 (0.55–1.46; P=.6468).
- COVID-19 adjusted — PFS-INV base case: HR 0.58 (95% CI 0.38–0.88; P=.0095). OS base case: HR 0.74 (0.43–1.25; P=.2587).
- Unmutated IGHV subgroup after matching (SEQUOIA ESS=93; CLL14 n=121): PFS-INV HR 0.63 (95% CI 0.39–1.03; P=.0652) base case and 0.61 (95% CI 0.37–0.99; P=.0475) COVID-19 adjusted.
- Sample size for SEQUOIA zanubrutinib: N=352, effective sample size 163 in the base case.

CITATION

Trial: SEQUOIA vs CLL14 MAIC (Munir, ICML 2025 P209)

File: Zanubrutinib Indirect Treatment Comparisons_Global_MRC-approved Aug 2025.pdf

Location: slides 53–59 (relative treatment effects slide 57, uIGHV slide 59)

Source date – Footer reads “Updated July 2025”; filename says MRC-approved August 2025.

“Acalabrutinib was non-inferior to ibrutinib in ELEVATE-RR. That is good enough for me.”

RESPONSE

“Zanubrutinib has demonstrated superior efficacy vs ibrutinib in a head-to-head randomized, clinical trial in patients with R/R CLL/SLL.”

“In the ELEVATE-RR trial, acalabrutinib demonstrated non-inferiority to ibrutinib in high-risk patients with CLL with del(17p)/TP53 and del(11q).”

From the October 2024 field handler: “Brukinsa is the only BTKi to show superiority vs Ibrutinib in phase 3 randomized clinical trial ‘Alpine’ while Acala PFS HR was 1.”

SUPPORTING DATA POINTS

- ALPINE, BRUKINSA (n=327) vs ibrutinib (n=325), 39.0 months mFU — All treated patients: 36-month PFS 64.9% vs 54.8% (HR: 0.68; 95% CI: 0.53, 0.86; 2-sided descriptive p=0.0011).
- ALPINE — Del(17p)/TP53 mutated: 36-month PFS 58.6% vs 41.3% (HR: 0.52; 95% CI: 0.33, 0.83; 2-sided descriptive p=0.0047).

CITATION

Trial: ELEVATE-RR (Byrd, J Clin Oncol 2021); ALPINE

File: Zanubrutinib Indirect Treatment Comparisons_Global_MRC-approved Aug 2025.pdf

Location: slide 5

Trial: ALPINE

File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf

Location: slides 11 and 8

Source date – ITC deck July 2025; field handler October 2024.

Access and cost

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OBJECTION 26 · ACCESS AND COST

“Why should I add BRUKINSA to the formulary when we already have acalabrutinib? Every addition costs the institution.”

RESPONSE

Acknowledge: “I understand that you are trying to maximize the use of the available formulary items / save cost for your patient and (or) institute.”

Probe / clarify: At the beginning what was your motive to add Acala while you had ibrutinib? What is your motive to add or delete a product? How do you see the current data of different BTKis?

Respond — Efficacy: The only BTKI to show superior efficacy against Ibrutinib in a phase 3 clinical trial. Showed superior efficacy in NMA published in Hemasphere 2022.

Respond — Number of indications: Brukinsa is approved in 5 different indications in FDA and 4 locally. Brukinsa is the only BTKi approved by EMA/FDA to treat MZL and FL.

Respond — Safety: Brukinsa demonstrated superior safety vs Ibrutinib in a randomized H2H clinical trial “ALPINE”. Brukinsa demonstrated superior safety profile vs Acalabrutinib in a huge 3rd party sponsored meta analysis included 61 different trials of different B-Cell malignancies.

Respond — Cost effectiveness: Brukinsa is more cost-effective VS Acalabrutinib if we used direct price comparison in NUPCO.

Respond — Flexibility of Dosing.

SUPPORTING DATA POINTS

- No figures are given on this slide. Neither the Hemasphere 2022 NMA, the 61-trial meta-analysis, nor the NUPCO price comparison is reproduced in any of the seven source files.

CITATION

Trial: ALPINE; Hemasphere 2022 NMA; 61-trial meta-analysis; NUPCO price comparison
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slides 12-14 (Objection #3)

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“In my country the tender price for BRUKINSA sits above acalabrutinib, so the pharmacy committee will not approve it.”

RESPONSE

Refer this question to Market Access. Pricing, tender and budget positions are set locally and are not covered by the approved clinical materials.

SUPPORTING DATA POINTS

- The pricing statement on file is: “Brukinsa is more cost-effective VS Acalabrutinib if we used direct price comparison in NUPCO.” NUPCO is the Saudi procurement body, so this speaks to Saudi Arabia rather than the wider Gulf.
- Pricing and tender positions in other Gulf markets are set locally by Market Access.

CITATION

Trial: –
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 14 (Objection #3)

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“Continuous therapy means an open-ended budget commitment. Fixed duration lets me forecast.”

RESPONSE

Refer this question to Market Access. Pricing, tender and budget positions are set locally and are not covered by the approved clinical materials.

The clinical sequencing point is the one available to you in the field: with a fixed-duration regimen you are using up one valid line of treatment for the patient without adding to the current efficacy.

SUPPORTING DATA POINTS

- The clinical point available in the approved materials is a sequencing one: “You are losing one valid line of treatment for the patient without adding to the current efficacy ... treatment sequencing.”

CITATION

Trial: –
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 7

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

Dosing and administration

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OBJECTION 29 · DOSING AND ADMINISTRATION

“AV is the first and only approved all-oral, chemo-free therapy in 1L CLL, and it is finished in just over a year.”

RESPONSE

BeOne lens: AV's cumbersome ramp-up, monitoring requirements, and rigid dosing schedule introduce complexity, uncertainty, inconvenience, and burden to both patients and providers.

Acalabrutinib and venetoclax have different dosing and monitoring requirements. Daily dosing and starting cycle: acalabrutinib BID from Cycle 1; venetoclax QD from Cycle 3. Dosing changes: acalabrutinib starts and stays at 100 mg; venetoclax starts at 20 mg, then 5-week ramp-up to 400 mg.

Lab monitoring: recommended for all patients during the 5-week ramp-up. TLS precautions for all patients: premedication, IV hydration, and frequent blood chemistry monitoring. More frequent monitoring is needed for patients at increased risk of TLS (bulky lymph nodes [≥5 cm], high absolute lymphocyte count, or low renal function).

Hospitalization is recommended for the first dose of venetoclax in patients with renal impairment due to the increased risk of TLS (fatal cases have been seen with a single 20 mg dose).

HCP prompts: How do you explain the different treatment options to patients? When a patient starts AV, what impact does it have on their job, family life, and carers vs. the impact felt from starting BRUKINSA? What work is involved for your center and support staff when starting a patient on AV? How does that compare with starting a patient on BRUKINSA?

KEY MESSAGE

BRUKINSA is a convenient, targeted oral monotherapy with a convenient dosing schedule and minimal treatment burden.

SUPPORTING DATA POINTS

- AV proof point as stated by AZ: AV is an all-oral, fixed-duration regimen that does not require IV infusion, therefore there are no ongoing treatment exposure, infusion-related reactions, or travel time required for infusion.
- BRUKINSA remains the only BTKi to offer 160 mg BID or 320 mg QD dosing options.
- Convenient, at-home once- or twice-daily dosing that fits easily into a patient's daily life and activities.
- No dose ramp-up or need for in-patient monitoring or hospitalization.

CITATION

Trial: AMPLIFY; CALQUENCE US PI Feb 2026; VENCLEXTA US PI Feb 2026; BRUKINSA US PI June 2025; BRUKINSA core data sheet V12.0
File: Objection Handler AMPLIFY.pptx
Location: slide 18 (AZ Claim 5)

Source date – April 2026 (1225-BRU-PRC-030).

“Twice a day is more of a burden for my patient than acalabrutinib’s schedule.”

RESPONSE

BRUKINSA remains the only BTKi to offer 160 mg BID or 320 mg QD dosing options.

Convenient, at-home once- or twice-daily dosing that fits easily into a patient’s daily life and activities. No dose ramp-up or need for in-patient monitoring or hospitalization.

From the field handler: BRUKINSA provides unmatched dosing flexibility. It’s the only BTKi that allows both once-daily and twice-daily dosing at home with no complicated ramp up or need for TLS monitoring, giving you the ability to tailor treatment schedules to each patient’s lifestyle and medical needs. With straightforward dose modifications, BRUKINSA enables simple personalisation of care.

KEY MESSAGE

BRUKINSA is a convenient, targeted oral monotherapy with a convenient dosing schedule and minimal treatment burden.

SUPPORTING DATA POINTS

- Comparator dosing as stated on the slide: acalabrutinib starts and stays at 100 mg BID from Cycle 1; venetoclax QD from Cycle 3, starting at 20 mg with a 5-week ramp-up to 400 mg.

CITATION

Trial: BRUKINSA US PI June 2025; BRUKINSA core data sheet V12.0; IMBRUVICA US PI Oct 2025; JAYPIRCA US PI Dec 2025
File: Objection Handler AMPLIFY.pptx
Location: slide 18

Trial: –
File: Objection hadler.pdf
Location: slide 6

Source date – April 2026, plus an undated file last modified January 2026.

“My patient is on a proton pump inhibitor. Does that not affect absorption?”

RESPONSE

The only statement on file is a single bullet: “Flexible dosing and not impacted by PPIs ‘In case of Capsule’.”

SUPPORTING DATA POINTS

- No pharmacokinetic data, no study and no figures accompany this statement anywhere in the seven source files.
- For context, the formulation is described elsewhere as “160mg BID or 320mg QD PO (80 mg capsules — tablet formulation in development)” (ZANU ALL INDICATIONS.pdf, “Zanubrutinib (BGB-3111) Overview” section).

CITATION

Trial: —

File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf

Location: slide 11

Source date — October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“BRUKINSA was third to market. Why would I move now?”

RESPONSE

“Despite coming third Brukinsa has demonstrated its activity across widest range of indications.”

From the same deck: “Brukinsa is approved in 5 different indications in FDA and 4 locally. Brukinsa is the only BTKi approved by EMA/FDA to treat MZL and FL.”

SUPPORTING DATA POINTS

- The April 2026 field guidance refers to “1,550 patients treated with BRUKINSA across five B-cell malignancies” (BRUKINSA EU SmPC, November 2025).
- Zanubrutinib is “approved in 70 countries/regions, including US, EU, UK, China, Australia, Canada, South Korea” and “more than 5,000+ patients enrolled in clinical trials in 35+ countries” (ZANU ALL INDICATIONS.pdf, “Zanubrutinib (BGB-3111) Overview” section, sourced to a BeiGene corporate presentation dated August 2023).

CITATION

Trial: –

File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf

Location: slides 11 and 14

Trial: –

File: ZANU ALL INDICATIONS.pdf

Location: section “Zanubrutinib (BGB-3111) Overview” (this file prints no slide numbers)

Source date – October 2024; supporting country and indication counts trace to an August 2023 corporate presentation.

“Trial populations are not my patients. What does the real-world evidence show?”

RESPONSE

Author conclusions (Flatiron): “In this retrospective observational real-world study, patients with CLL receiving 1L BTK inhibitor monotherapy with zanubrutinib had significantly prolonged rwTTNT, rwTTD, and rwOS compared with those receiving ibrutinib. Further analyses should be performed at longer follow-up to confirm and expand findings.”

SUPPORTING DATA POINTS

- Flatiron, 1L CLL comparative effectiveness (Jacobs, EHA 2025 PS1578) — real-world time to next treatment: median rwTTNT not reached for zanubrutinib and acalabrutinib and 38.2 months (95% CI, 33.2, 42.3) for ibrutinib. Adjusted HR vs ibrutinib 0.59 (95% CI, 0.44, 0.79; $P=0.0004$); vs acalabrutinib 0.86 (95% CI, 0.64, 1.16; $P=.3237$).
- Flatiron — real-world time to treatment discontinuation: median rwTTD not reported for zanubrutinib vs 43.7 months (95% CI, 34.4, NR) for acalabrutinib and 21.9 months (95% CI, 18.3, 25.7) for ibrutinib. Adjusted HR vs ibrutinib 0.56 (95% CI, 0.44, 0.72; $P<.0001$); vs acalabrutinib 0.89 (95% CI, 0.69, 1.15; $P=.3827$).
- Flatiron — real-world overall survival: median rwOS not reached for any treatment. Adjusted HR vs ibrutinib 0.46 (95% CI, 0.28, 0.76; $P=.0024$); vs acalabrutinib 0.80 (95% CI, 0.49, 1.33; $P=.3931$).
- TriNetX retrospective real-world study in 1L CLL: 51% lower risk of new-onset atrial fibrillation/flutter vs. acalabrutinib (RR: 0.49, 95% CI: 0.27, 0.91; $p=0.020$); 27% lower risk of major cardiovascular AEs vs. acalabrutinib (RR: 0.73; 95% CI: 0.58, 0.93; $p=0.009$). Risks of new-onset hypertension, arrhythmia, new-onset heart failure, myocardial infarction, conduction block, and stroke were similar between groups. (Tang, Blood 2024.)

CITATION

Trial: Flatiron 1L CLL comparative effectiveness (Jacobs, EHA 2025 PS1578)
File: ZANU ALL INDICATIONS.pdf
Location: section “1L CLL Comparative Effectiveness (Flatiron)” (this file prints no slide numbers)

Trial: TriNetX (Tang, Blood 2024)
File: Objection Handler AMPLIFY.pptx
Location: slide 14

Source date — EHA 2025 and April 2026.

“You keep talking about CLL. What about my Waldenström’s patients?”**RESPONSE**

These are Waldenström's macroglobulinemia data, not CLL — the two indications are answered from separate approved materials.

Zanubrutinib is approved for WM by SFDA, FDA and EMA. Acalabrutinib is not approved for WM.

The pivotal trial is ASPEN, a Phase 3 randomized head-to-head study against ibrutinib, at a median follow-up of 69.8 months. There is no Phase 3 trial of acalabrutinib in WM.

NCCN guidelines 2025 give zanubrutinib a Level 1 recommendation in primary therapy for WM/LPL and in previously treated WM/LPL.

In ASPEN cohort 1, zanubrutinib vs ibrutinib showed a reduction in the risk of atrial fibrillation/flutter (2% vs 15%, $P=0.0004$); lower rates of major bleeding (0.3 vs 0.6 events/100 person-months), diarrhea (21% vs 32%), and hypertension (11% vs 16%); grade ≥ 3 infection rates were similar, despite higher rates of neutropenia with zanubrutinib; and fewer AEs leading to death or treatment discontinuation with zanubrutinib.

KEY MESSAGE

Zanubrutinib is the only second-generation BTK inhibitor approved in WM, on the strength of a head-to-head Phase 3 trial.

SUPPORTING DATA POINTS

- Zanubrutinib is approved for WM by SFDA, FDA and EMA. Acalabrutinib is “Not approved for WM”.
- Pivotal trial: ASPEN (Phase 3, randomized, head-to-head vs ibrutinib), mFU 69.8 months. The comparator column on the same slide records “No Phase 3 trial in WM, mFU = 63 months”.
- Efficacy figures printed on the slide: ORR = 96%, VGPR = 40.2%; and ORR = 92%, VGPR = 29.5%; 60 months PFS 74.8% and 58%. MYD88WT WM: enrolled / not reported.
- ASPEN cohort 1 safety, zanubrutinib vs ibrutinib as reported in the same file: “A reduction in the risk of atrial fibrillation/flutter (2% vs 15%, $P=0.0004$); lower rates of major bleeding (0.3 vs 0.6 events/100 person-months), diarrhea (21% vs 32%), and hypertension (11% vs 16%); grade ≥ 3 infection rates were similar, despite higher rates of neutropenia with zanubrutinib; fewer AEs leading to death or treatment discontinuation with zanubrutinib.”
- NCCN guidelines 2025: Level 1 recommendation in primary therapy for WM/LPL and in previously treated WM/LPL.

CITATION

Trial: ASPEN (Tam, Blood 2020; Tam, ASCO 2020 abstract 8007)

File: Zanubrutinib data in WM.pdf

Location: slides 3-6, and the ASPEN cohort 1 safety summary slide

Source date – Code 0922-BGB-3111-MRC-079. Footers on this file read variously “Updated February 2024”, “Updated April 2024” and “Updated September 2024”, while the guideline slides cite NCCN 2025.

“What do the guidelines actually say about where BRUKINSA sits in CLL?”

RESPONSE

Refer this question to Medical Information. Requests for information outside the approved materials must be unsolicited and must follow the Medical Information Request (MIR) process.

SUPPORTING DATA POINTS

- NCCN CLL/SLL guidelines appear in the approved materials as reference citations: “NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®) for Chronic Lymphocytic Leukemia/Small Lymphocytic Leukemia V2 2026”, and “V.3.2025” in the real-world evidence section of ZANU ALL INDICATIONS.
- Guideline recommendation slides in the approved set cover WM/LPL, where NCCN 2025 gives zanubrutinib a Level 1 recommendation in both primary and previously treated disease.
- For a CLL guideline-positioning slide, request one from Medical Information.

CITATION

Trial: NCCN CLL/SLL V2 2026 (reference list)
File: Objection Handler AMPLIFY.pptx
Location: slide 13

Source date – April 2026 (1225-BRU-PRC-030).

“If I start with BRUKINSA, what do I have left when the patient progresses?”

RESPONSE

Refer this question to Medical Information. Requests for information outside the approved materials must be unsolicited and must follow the Medical Information Request (MIR) process.

SUPPORTING DATA POINTS

- The sequencing content in the approved materials is a single line, used against fixed duration rather than about what follows BRUKINSA: “You are losing one valid line of treatment for the patient without adding to the current efficacy ... treatment sequencing.”
- For a post-BTKi sequencing discussion, route the question to Medical Information.

CITATION

Trial: –
File: GCC BRUKINSA CLL Objection Handler_V5.0 (x2 fixed duration objections) - APPROVED.pdf
Location: slide 7 (Objection #1)

Source date – October 2024 (deck cover); code 0324-BRU-PRC-052, April 2024.

“Pirtobrutinib is coming, and BTK degraders after that. Why commit to a covalent BTK inhibitor now?”

RESPONSE

Refer this question to Medical Information. Requests for information outside the approved materials must be unsolicited and must follow the Medical Information Request (MIR) process.

SUPPORTING DATA POINTS

- Pirtobrutinib appears in the approved materials once, as a dosing comparison reference: “JAYPIRCA (pirtobrutinib). US PI, December 2025”, alongside the acalabrutinib, ibrutinib and BRUKINSA prescribing information used for the dosing comparison.
- BRUKINSA remains the only BTKi to offer 160 mg BID or 320 mg QD dosing options.
- For questions on non-covalent BTK inhibitors or BTK degraders, route to Medical Information.

CITATION

Trial: JAYPIRCA (pirtobrutinib) US PI, December 2025
File: Objection Handler AMPLIFY.pptx
Location: slide 18 (AZ Claim 5)

Source date – April 2026 (1225-BRU-PRC-030).